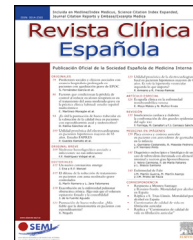




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CONSENSUS DOCUMENT

Strategic framework for the Spanish Society of Internal Medicine. 2025-2029. Executive summary on guaranteeing person-centered care in a high value National Health System



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KEYWORDS

Internal Medicine;
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Abstract

Aims: To design a strategic framework for the policy and activity of the Spanish Society of Internal Medicine (SEMI).

Methods: Expert consensus. Participation through expert committees and consultation of internists on the proposed strategic lines.

Results: The SEMI's motto was changed to "comprehensive care for patients." Seven strategic lines were defined: Contribute to the transformation of the health system; participate in university teaching; guarantee comprehensive healthcare for patients; provide internal medicine residents and internists with the skills to practice in 21st century healthcare; promote research and the generation of scientific evidence in internal medicine; incorporate technological innovation and digital tools; position the SEMI as a model institution for healthcare policy.

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◇ Listed in Appendix A.

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Conclusions: SEMI seeks to be a key player in the transformation of the Spanish healthcare system.

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PALABRAS CLAVE

Medicina Interna;
Reflexión estratégica;
Sociedad española de
medicina interna;
Calidad;
Eficiencia

RESUMEN EJECUTIVO DE LA REFLEXIÓN ESTRATÉGICA DE LA SOCIEDAD ESPAÑOLA DE MEDICINA INTERNA 2025-2029 para garantizar la asistencia integral a la persona enferma en un Sistema Nacional de Salud de alto valor

Resumen

Objetivos: Diseñar un marco que oriente la política y actividad de la Sociedad Española de Medicina Interna (SEMI).

Material y métodos: Consenso de expertos. Participación mediante comités de expertos y consulta a los socios de las propuestas de líneas estratégicas.

Resultados: Se modificó el lema de la SEMI por «la asistencia integral a la persona enferma», definiendo siete líneas estratégicas: Contribuir a la transformación del sistema sanitario; participar en la docencia universitaria; garantizar una asistencia integral a la persona enferma; dotar al residente de MI y al internista de las competencias para ejercer en la sanidad del siglo XXI; fomentar la investigación y la generación de evidencia científica en MI; incorporar la innovación tecnológica y las herramientas digitales; posicionar a la SEMI como una institución de referencia para la política sanitaria.

Conclusiones: La SEMI se propone como un actor clave en la transformación del sistema sanitario español.

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Introduction

In 2005, the European Federation of Internal Medicine published a strategic positioning report¹ and in 2011, the Spanish Society of Internal Medicine (SEMI) published a strategic orientation report.² More than ten years after the SEMI's strategic orientation report, the SEMI Board of Directors decided to launch a strategic framework. Main drivers for this strategic framework are the crisis of the Spanish National Health System (SNHS)³ and the central role and commitment of the Internal Medicine (IM) with its sustainability and quality (Azores Declaration).⁴ Other important factors driving this reflection are the changes that the evolution of scientific knowledge and the emergence of disruptive technologies such as digitalization and generative artificial intelligence are causing in healthcare organization and processes.⁵

The main objective of this project was to design a framework to guide the policy and actions of the SEMI, internal medicine departments (IMD), and their partners that influences patients, industry, other scientific societies, and public administrations. The strategic framework is approached from a global perspective and then identifies and defines the activities, goals, and budgets for each specific project, depending on the resources the SEMI can devote to them and the priorities estab-

lished at any given time by the respective boards of directors.

The specific objectives of the strategic framework are as follows:

- To elaborate a strategic analysis of the current situation of IM and its future projections within the NHS.
- Identify the strengths, weaknesses, opportunities, and threats (SWOT) for internists, IMDs, and the SEMI in the environmental and internal analysis.
- Develop strategic lines to correct weaknesses, address threats, maintain strengths, and exploit opportunities (CAME) for the three agents identified: internists, IMDs, and the SEMI.
- Define the profile of internists in future scenarios.
- Propose policies to drive the necessary changes in the structure, organization, operation, and quality standards of IMDs.
- Identify the role to be played by the SEMI in the development and implementation of the policies proposed in the strategic framework.

This article is an executive summary of the full strategic framework document, which can be found in the [Supplementary material](#).

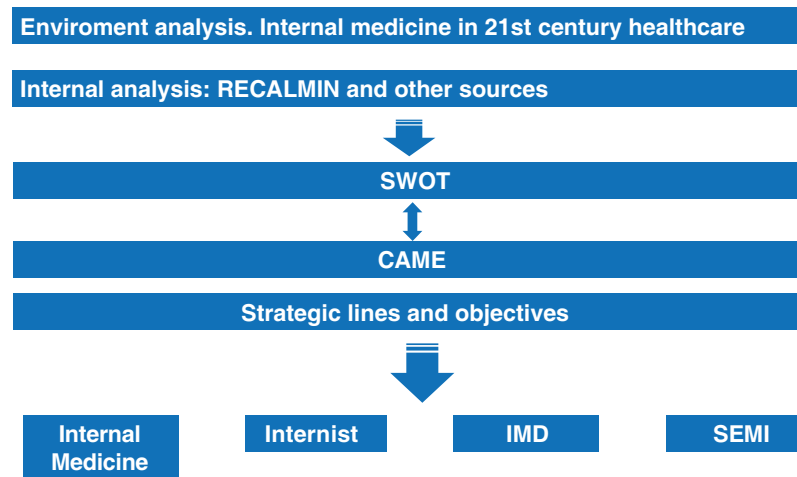


Figure 1 Stages of the Spanish Society of Internal Medicine's strategic framework process.

SWOT: Strengths, weaknesses, strengths, and opportunities. CAME: correct weaknesses, adapt to threats, maintain strengths, and exploit opportunities; IMD: Internal medicine departments; SEMI: Spanish Society of Internal Medicine.

Methods

Working groups and member opinions

The method used was expert consensus. To this end, the SEMI Board of Directors created a Steering Committee comprising members of the Standing Committee of the Board of Directors with representation from members of the Board of Directors, members of the SEMI working groups, and internists with management experience. The Steering Committee made the final decisions on the various components of the strategic framework. The Steering Committee's work was reinforced by an "Enlarged Committee" that included some former SEMI presidents, SEMI representatives on the National Specialty Commission, and other internists with proven experience in IMD management. Given the importance of young people's opinions in the reflection process, a young internists group was created. Likewise, a survey was sent to IM residents to gather their opinions on the SEMI's operations and activities. The composition of the groups can be consulted in the full strategic framework report, which is included as [Supplementary material](#) to this position paper. Finally, in an attempt to encourage maximum participation, IMD managers and members were asked to send their contributions and proposals on the strategic lines, objectives, and goals. The drafts of the different report sections were prepared by technical management external to the SEMI (Institute for Healthcare Improvement Foundation). The strategic framework process began in November 2023 and ended in July 2024.

Strategic framework development process

The reflection process followed the steps summarized in [Fig. 1](#). At each stage of the process, comments on the drafts prepared by the technical management were requested from members of the various committees. The different parts of the report were finalized in face-to-face meetings in which the most controversial aspects were discussed.

Results

The full report is included in the [Supplementary material](#). A SWOT analysis was performed based on the environmental and internal analysis, as shown in [Table 1](#). In regard to the components of the SWOT analysis, strategies and activities were identified to correct weaknesses, address threats, maintain strengths, and exploit opportunities (CAME matrix) ([Table 2](#)).

SEMI motto

Comprehensive care and continuity of care were considered two of the main concepts that guide IM and the operation of IMDs. They represent an alternative to the current fragmented model that does not respond to most health needs of the current population and, therefore, is neither efficient nor sustainable. The pressures caused by demographics and the need to control healthcare costs have driven a significant growth of "generalism" in healthcare systems. It has led to the creation of figures such as the "hospitalist" in the United States of America or the "acute medicine doctor" in the United Kingdom, for example.^{6,7} On the other hand, focusing IMD care on "patients" implies paying attention to principles that incorporate holistic dimensions of the individual, including social determinants.⁸ Given the importance of both concepts—comprehensive care and person-centered care—this strategic framework has led to a change in the SEMI's motto from the previous "a comprehensive vision of patients" to the new "comprehensive care for patients."

SEMI's strategic lines. IM's contribution to the "five-fold objective"

The environment analysis concluded that a focus on health, the inclusion of citizens and patients as managers of their health, and the disruptive changes that digitalization will cause in healthcare organization and management are likely

Table 1 Spanish Society of Internal Medicine Strategic framework. SWOT Matrix (summary).

SWOT	
Weaknesses	Threats
- Insufficient weight in healthcare policy - Little investment by the pharmaceutical industry - Precarious presence of internists in universities - Insufficient training of internists on biomedical research and publications - Poor integration of IMD units such as home hospitalization and lack of coordination with emergency services. - Room for improvement in implementation of comanagement, chronic patient care programs, rapid diagnosis, etc.	- Fragmented healthcare system with significant resistance to change - Insufficient presence in undergraduate education - Changes in citizens' expectations and demands - Creation of new specialties
Strengths	Opportunities
- Capacity for comprehensive care and continuity of care - Willingness for teamwork - Adaptability and versatility for a comprehensive approach to patients and for the organization of care (management) - Hospitalists	- Contribution to NHS' efficiency - Leadership, together with PC, on chronicity - Address the health implications of climate change - Optimization of healthcare with digitization - Improvements in public health coordination

Table 2 Spanish Society of Internal Medicine Strategic Framework. CAME Matrix (summary).

Came	
Correct weaknesses	Adapt to threats
- Strengthen alliances with scientific societies - Promote the implementation of proposals for the "hospital of the future" - Strengthen information systems on IMD activity and results - Make our potential visible	- Demand the approval of the training program proposed by the National Specialty Commission (CNE, for its initials in Spanish) - Boost the presence of internists in universities - Promote the SEMI's proposals on the "hospital of the future" - Provide training and promote digitization - Strengthen relations with patient and family associations
Maintain strengths	Exploit opportunities
- Establish the strategic line of action of comprehensive care and continuity of care as the main contribution of IM to the healthcare system - Strengthen the implementation of care tools and devices (ultrasound, day hospitals, specialized clinics, rapid diagnostic units, home hospitalization, comanagement, etc.) - Make IM's ability to adapt and respond to emerging problems and its capacity for continuous innovation more visible	- Better position our important role in chronicity, continuity of care, comanagement, climate change, public health coordination, different care departments, and more. - Use artificial intelligence as a tool to facilitate and improve daily assistance - Strengthen relations with internal medicine societies in Europe, Latin America, and the USA and with societies in other specialties - Strengthen alliances with nursing departments

the three most important vectors of change in healthcare systems.

The reorientation of healthcare systems toward health is no simple feat; indeed, it entails a Copernican revolution in the understanding of the role of healthcare in health. In 2008, Dr. Donald Berwick, director of the Institute for Healthcare Improvement, and his collaborators proposed a three-fold aim for the US healthcare system: care (improving the patient experience), health (improving the health of the population), and cost (reducing the per-person cost of healthcare).⁹ Subsequently, Porter and Lee proposed a strategy to "fix" the healthcare system that synthesized Berwick et al.'s three objectives into "improve value,"¹⁰ a concept that was adopted by the SEMI as a strategy for change to adapt IM to the challenges of the 21st century.¹¹ As an expansion of the objectives outlined by Berwick et al., a "four-fold objective" has been proposed to take into account professionals' experience.¹² Lastly, equity and quality of the healthcare system have been added as a "five-fold objective."^{13,14} Taking all these objectives as the most relevant for health systems, the "five-fold objective" (five complementary objectives in one strategy) is expressed as:

- Improve the population's health.
- Improve patients' experience.
- Improve professionals' experience.
- Increase the equity and quality of the system.
- Increase the efficiency of the system.

The strategic lines formulated are:

- Strategic line 1. Contribute to the healthcare system's transformation toward a model of care that more effectively achieves the "five-fold objective."
- Strategic line 2. Participate in university teaching in order to incorporate comprehensive care of people as an essential concept of medical knowledge.
- Strategic line 3. Strengthen the SEMI's activities related to the central mission of IM: guaranteeing comprehensive care for patients (comanagement, pluripathological and chronic complex patients, etc.).
- Strategic line 4. Ensure IM residents develop the IM-specific and multidisciplinary skills necessary to practice IM in 21st century healthcare and strengthen training and continuing professional development.
- Strategic line 5. Promote research and the generation of scientific evidence in IM.
- Strategic line 6. Incorporate technological innovation and digital tools, contributing to the digital transformation of the healthcare system from the IM knowledge area.
- Strategic line 7. Position SEMI as a model for decision making in health policy and communicate the contribution of IM to society.

Strategic objectives, specific objective, and goals

Strategic objectives have been established for each strategic line and, within them, tentative goals have been set in order to achieve them. The list of strategic lines for these objectives and goals is shown in [Table 3](#).

Table 3 Strategic lines, strategic and specific objectives, and goals. SEMI's strategic framework. 2025–2029.

Strategic objectives	Specific objectives	Goals (2024/2025–2029)
Strategic line 1. Contribute to the healthcare system's transformation toward a model of care that more effectively achieves the "five-fold objective." The SEMI should drive a change in the current care model to one based on comprehensive patient care. This fundamental objective underlies the rest of the SEMI's strategic lines and objectives for the 2025–2029 period	Change SEMI's motto to "comprehensive care for patients"	In the presentation of the strategy (2024) 2024/1Q 2025
	Establish alliances with primary care societies and patient associations to develop a proposal for a change in the care model summarized in a few key points Communications campaign regarding the comprehensive care model and presentation to public administrations (national government and autonomous communities)	2024/1Q 2025
<i>1.1. Improve the population's health</i> Promote positive changes in lifestyle and in patients'/caregivers' knowledge and management of their disease	Develop educational and informational activities in collaboration with patient associations, especially activities aimed at pluripathological and chronic complex patients Facilitate tools for IMDs to implement health promotion actions through the Working Group on Health Education Create a platform of educational videos for patients and their families	≥2 educational activities/year (2025–2029) 2024–2029 2024–2029
Pay attention to the health consequences of climate change	Develop a specific line of work on IM and climate change Develop best practice recommendations to contribute to environmental sustainability and reducing the impact of the ecological transition on health	2024–2029
<i>1.2. Improve patients' experiences</i> Incorporate PREM and PROM, establishing alliances with patient associations	Develop PROMs for pluripathological patients and other conditions prevalent in IM (e.g., complex chronic patients - heart failure; COPD, etc.) Develop PREMs of IMDs	2025–2029 2025–2026 2026
Education for patients and caregivers. Promote health education with the figure of the expert patient and virtual classrooms Incorporate PPIs, actively involving the people attended to in the research process	Develop, with the collaboration of patient associations, a virtual classroom focused mainly on pluripathological/chronic complex patients and their caregivers	2025–2029
<i>1.3. Improve professionals' experience</i> Spearhead the approval of a training program for the specialty Update the general and specific skills and recertification criteria, establishing homogeneous criteria with the rest of the medical specialties through FACME	In collaboration with FACME, condemn the unjustified delay in the program's approval and demand its immediate approval Update the skills and criteria for recertification in internal medicine (Board of Directors, National Specialty Commission, and Working Group on Education)	2024 2025

Table 3 (Continued)

Strategic objectives	Specific objectives	Goals (2024/2025–2029)
Collaborate with FACME and CGCOM in promoting a change in physicians' status within the NHS that favors a collaborative relationship and managerial autonomy as opposed to the current model of top-down control <i>1.4. Increase equity and quality</i> <i>1.4.1. Increase equity</i> Strengthen coordination with federated societies by creating an "Interterritorial SEMI Council" <i>1.4.2. Improve patient care</i> Continuing education aimed at improving patient care (multidisciplinary and specific skills) Incorporating information on the psychosocial dimensions of health (educational level, self-perception of health, loneliness, economic hardship, etc.) into the medical record <i>1.4.3. Improve the quality of care provided in IMDs</i> <i>1.5. Improve efficiency</i> Promote, design, and implement new clinical care models in IM	Strengthen the IM recertification program Create a line of work for the "advanced training" of internists as part of enacting RD Royal Decree 639/2015, which regulates accreditation diplomas and advanced accreditation diplomas, in collaboration with other scientific-medical societies and FACME	2024–2029 2025–2029
		2024–2029
	Establish joint actions (SEMI Board of Directors, autonomous community-based societies) at the regional level to promote comprehensive care models for patients, incorporating the objectives of these strategic lines at the regional level	2025–2029
	Prepare a biannual report (coinciding with RECALMIN editions) on differences in resources (e.g. internists) as well as situations of inequity in IM healthcare	2025–2029
	Analyze the interterritorial situation and differences of certain IM programs (continuous care, comanagement, home hospitalization, programs for pluripathological patients, etc.)	2025–2029
		2025–2029
	Training on: Comprehensive assessment/management of frailty Person-centered care Shared decision making Effective communication with patients and family members Patient safety	2025–2029
	Review and update IMD standards	2025
	Strengthen the certification and recertification processes of IMDs and functional units/specialized IM clinics	2025–2029
	Preferential areas of focus:	2025–2029

Table 3 (Continued)

Strategic objectives	Specific objectives	Goals (2024/2025–2029)
This involves activities included in other strategic lines and objectives: Having information available, researching, and generating evidence (evaluating efficiency, quality, etc.) on new clinical care models in IM Create a system for sharing experiences and scaling up the most successful ones Develop/update quality standards and certification systems for these models Train internists and IMDs on the organization and management of new models Disseminate and support the implementation of the new models	Coordination of care for patients admitted to the hospital (quality standards and guarantee of continuous care)* Care programs for pluripathological patients** Care programs for patients with systemic autoimmune diseases Comanagement	
	Home hospitalization	
	Teams of internists in low-complexity hospitals with support from subspecialists at the referral hospital	
	Rapid diagnostic or rapid resolution consultations	
Promote organizational designs of IMDs in which collaboration with other departments in managing multimorbidity (pluripathology, complex chronic patients) is considered a relevant area of action, developing structured multidisciplinary units	Internist as referring physicians and the gateway to the "hospital" level of care (referred from PC for diagnosis) *In collaboration with emergency and intensive care medicine departments and societies; **In collaboration with PC societies and, if possible, other societies (geriatrics, palliative care, etc.) Design strategic alliances with other specialties, including primary care, that include objectives	2025
	Define the characteristics and criteria for the certification of interdisciplinary units or teams in collaboration with other scientific societies Inter- and multidisciplinary units for diabetes, COPD, heart failure, obesity, etc. Tumor committees/multidisciplinary cancer units	2025–2029
		2024–2029
Develop/update IMD performance indicators and benchmarking systems 2. Participate in university teaching in order to incorporate comprehensive care of people as an essential concept of medical knowledge		

Table 3 (Continued)

Strategic objectives	Specific objectives	Goals (2024/2025–2029)
Establish alliances with universities/medical schools so that medical degree programs have an comprehensive, holistic approach, paying more attention to education on general pathology and internal medicine	Create an <i>ad hoc</i> group (Board of Directors, National Specialty Commission, Working Group on Education) to collaborate with the National Conference of Medical School Deans and medical schools	2025–2029
At the undergraduate level, carry out specific actions to attract talent and highlight the specialties' appeal	Proposal to add an evaluation of the non-technical skills considered most critical to the current entrance exam for medical residents (MIR, for its initials in Spanish) through a situational judgment test	2024–2025
<i>Strategic line 3. Strengthen the SEMI's actions related to the central mission of IM: guaranteeing comprehensive patient care (comanagement, pluripathological and complex chronic patients, etc.)</i>		
This involves activities included in other strategic lines and objectives:	Preferential areas of focus:	2025–2029
Having information available, researching, and generating evidence on the structure, resources, and activity of IM	Care programs for pluripathological patients**	
Create a system for sharing experiences and scaling up the most successful ones	Comanagement	
Develop/update quality standards and certification systems	Home hospitalization	
Train internists and IMDs on the organization and management of IMDs	Teams of internists in low-complexity hospitals with support from subspecialists at the referral hospital Rapid diagnostic or rapid resolution consultations Internist as referring physicians and the gateway to the "hospital" level of care (referred from PC for diagnosis)	
<i>Strategic line 4. Strengthen education and continuing professional development, contributing to upholding and updating specific professional skills for IM and providing internists with the multidisciplinary skills that the transformation of the healthcare system will require</i>		
Ensure IM resident physicians develop the specific and multidisciplinary skills necessary to practice IM in 21 st century healthcare		
Collaborate on training resident physicians on multidisciplinary skills		

Strategic objectives	Specific objectives	Goals (2024/2025–2029)
	Implement a training plan on multidisciplinary skills for IM residents and for internists Deontology and professionalism Bioethics. Ethics of efficiency Holistic concept of health. Health determinants. Social determinants Epidemiology Person-centered care. Person-centered prescribing. Shared decision making. Effective communication with patients and family members Clinical management. Leadership, change management, management of multidisciplinary teams Quality and patient safety Clinical research Oral communication (scientific presentations), research, and biomedical publications skills Use of remote simulation tools (serious games) for developing multidisciplinary competencies	2025–2029
Increase the participation of young physicians and residents in the SEMI		2025–2029
<i>Strategic line 5. Promote research and the generation of scientific evidence in internal medicine</i>		
Develop specific lines of research for “core” IM activities	Preferred areas of interest for health outcomes research: Care programs for pluripathological/chronic complex patients Systematic care of patients admitted to the hospital (alert and rapid response systems, etc.) Comanagement Home hospitalization Integration of internists into inter- and multidisciplinary units E-consultations; Telemedicine	2025–2029
Conduct of multicentric projects by working groups		2025–2029
Promote the dissemination of evidence generated by specific lines of research through our own publications and especially through the <i>Revista Clínica Española</i>		
<i>Strategic line 6. Incorporate technological innovation and digital tools as a core concept, contributing to the digital transformation of the healthcare system from the IM area of knowledge</i>		

Table 3 (Continued)

Strategic objectives	Specific objectives	Goals (2024/2025–2029)
Contribute to SEMI members' digital skills and the incorporation of these tools in IMD activities	Digital skills training (possible collaboration with technology partners) Define the digital skills that internists should have Digital skills training	2025 2025–2029 2025–2029
Develop projects for the application of digitalization technologies, including AI, to improve the quality and efficiency of IMDs		
Promote telemedicine	Train internal medicine residents and internists in telemedicine skills (e-consultation, remote monitoring, etc.) Develop quality and safety standards for IM telemedicine	2025–2029
Promote data culture as an incitement for developing the medical record 2.0, a digital and interoperable medical record		2025–2029
Promote an open innovation strategy, involving other healthcare professionals, bioinformaticians, bioengineers, patients, and digital health technical professionals		
<i>Strategic line 7. Position SEMI as a model for decision making in health policy. Communicate IM's contribution to the sustainability of the healthcare system to society</i>		

Table 3 (Continued)

Strategic objectives	Specific objectives	Goals (2024/2025–2029)
Encourage communication and dissemination of the actions and role of IM in order to promote greater knowledge and awareness in society, especially regarding the critical role of IM in the management of chronic complex and pluripathological patients and in the continuity of care	Manifesto on the need for a change in the healthcare model and the role of IM in this new model External communication campaign on the value contributions of IM: Comprehensive, person-centered care and “generalism” are two concepts provided by IM that are necessary for overcoming the current crisis in the healthcare system Systematic care programs for pluripathological patients. The most profitable investment that the National Health System should make	
Strengthen and revalue SEMI's Board of Directors	Analyze the registration of the working groups on health education, clinical management, and digital medicine on an innovation platform	2025–2029
Strengthen alliances with other scientific societies and especially with international internal medicine scientific societies	Creation of a working group on cooperation for health development	2025–2029
PC: Primary care; AC: Autonomous Communities; CGCOM: General Council of Official Medical Colleges; COPD: chronic obstructive pulmonary disease; FACME: Federation of Spanish Scientific-Medical Associations; IM: Internal Medicine; PPI: Patient and Public Involvement; PREM: Patient Reported Experience Measures; PROM: Patient Reported Outcomes Measures; SEMI: Spanish Society of Internal Medicine; IMD: Internal Medicine Department.		

Conclusions

Through this strategic framework, the SEMI seeks to position itself as a key player in the transformation of the Spanish healthcare system, adapting to 21st century challenges and focusing its efforts on more comprehensive and personalized care.

The proposed strategic lines seek to contribute to the transformation of the healthcare system, university teaching, the guarantee of comprehensive care, the development of residents' skills, research, technological innovation, and the positioning of SEMI as a model in healthcare policy. All of this forms part of achieving the "five-fold objective": improving the population's health and patients' and professionals' experience, increasing the equity and quality of the system, and improving the efficiency of the healthcare system.

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Conflicts of interest

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Appendix A. Members of SEMI's strategic framework committees

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- Members of working groups and experts
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Appendix B. Supplementary data

Supplementary material related to this article can be found, in the online version, at doi:<https://www.fesemi.org/sites/default/files/documentos/1042.pdf>.

References

1. Bauer W, Schumm-Draeger PM, Koebberling J, Gjoerup T, García Alegría JJ, Ferreira F, et al., The EFIM Working Group on Political Issues in Internal Medicine in Europe. Political issues in internal medicine in Europe. A position paper. *Eur J Intern Med*. 2005;16:214–7, doi:10.1016/j.ejim.2005.02.003.
2. García-Alegría J, Conthe-Gutiérrez P, por la Junta Directiva de la Sociedad Española de Medicina Interna. Orientación estratégica de la Sociedad Española de Medicina Interna. *Rev Clin Esp*. 2011;211:46–51.
3. Los Retos del Sistema Nacional de Salud en la Próxima Legislatura. 2023-7. IMAS. CGCOM. FACME. May, 2023. Available from: [Retos_SNS_2023-2027.pdf](https://www.fesemi.org/sites/default/files/documentos/Retos_SNS_2023-2027.pdf)imasfundacion.es).
4. Declaración de las Azores. Sociedad Portuguesa de Medicina Interna – Sociedad Española de Medicina Interna. 29-04-2018. Available from: Declaración de las Azores ([fesemi.org](https://www.fesemi.org)).

5. Filomena P, Sebastian K, Kati K, Tiffany L, Ismael SC, EFIM TelemedicineInnovative Technologies and Digital Health Working Group. Telemedicine in internal medicine: a statement by the European Federation of Internal Medicine. *Eur J Intern Med.* 2023;112:138–9, doi:10.1016/j.ejim.2023.02.021. PMID:36872139.
6. Wachter RM, Bell D. Renaissance of hospital generalists. *BMJ.* 2012;344:e652, doi:10.1136/bmj.e652. PMID: 36872139.
7. Temple RM, Kirthi V, Patterson LJ. Is it time for a new kind of hospital physician? *BMJ.* 2012;344:e2240, doi:10.1136/bmj.e2240. PMID: 22491697.
8. Chimeno Viñas MM, Pérez-Martínez P. Social determinants of health and their impact on internal medicine. *Rev Clin Esp (Barc).* 2024;224:398–9, doi:10.1016/j.rceng.2024.05.003. PMID: 38763465.
9. Berwick DM, Nolan TW, Whittington J. The triple aim: care, health and cost. *Health Aff.* 2008;27:759–69.
10. Porter ME, Lee TH. The strategy that will fix health care. *Harvard Business Review*; 2013. October.
11. Varela J, Zapatero A, Gómez-Huelgas R, Maestre A, Díez-Manglano J, Barba R. Por una Medicina Interna de alto valor. In: Sociedad Española de Medicina Interna; 2019.
12. Sikka R, Morath JM, Leape L. The quadruple aim: care, health, cost and meaning in work. *BMJ Qual Saf.* 2015;24:608–10.
13. Nundy S, Cooper LA, Mate KS. The quintuple aim for health care improvement: a new imperative to advance health equity. *JAMA.* 2022;327:521–2, doi:10.1001/jama.2021.25181. PMID: 35061006.
14. Dzau VJ, Mate K, O’Kane M. Equity and quality-improving health care delivery requires both. *JAMA.* 2022;327:519–20, doi:10.1001/jama.2022.0283. PMID: 35060998.